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Infectious Diseases
Hospital Epidemiology

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Division of Dockets Management (HFA-305)
Food and Drug Administration
5630 Fishers Lane, Rm. 1061
Rockville, MD 20852

Re: Docket No. FDA-2015-D-3719 for “Draft Guidances Relating to the Regulation of Human Cells, Tissues, and Cellular and Tissue-Based Products; Rescheduling of Public Hearing; Request for Comments”

Dear Sir or Madam:

My name is John Samies, and I am a Board Certified Infectious Diseases specialist and certified wound specialist at the Regional Medical Center in Orangeburg, South Carolina. I received my medical degree from Hahnemann Medical College in Philadelphia and have been in practice for more than 30 years. My focus on the draft guidance documents relates primarily to HCT/Ps intended for use in wound healing.

It is estimated that, at any given time, 1% of the population is suffering from some form of chronic wound. These wounds have profound costs both financially and on a personal and emotional level in our society. It is therefore of utmost importance that patients and health care providers have proven products. Unfortunately, products with largely unsubstantiated claims are emerging. I am bombarded by marketing of new products which imply more than homologous use yet assert their status under Section 361. Additionally, the adverse event reporting for Section 361 products is limited only to reporting related to transmission of infectious disease – and does not include any other adverse outcomes.

Section 361 HCT/Ps have an important place in the toolkit available to wound care specialists. As examples, cadaver dermis and amniotic tissue are well-suited for use as wound coverings, and the regulations in Part 1271 provide sufficient oversight for these products. The objective of these regulations is to prevent the transmission of infectious diseases from donors to recipients. However, when claims of complex biological interactions are made for HCT/Ps - such as modifying wound healing - the Part 1271 regulations are insufficient alone, since they are **NOT** intended to assure the safety and efficacy of these products beyond the transmission of infectious disease. For example, how does one assess the oncogenic potential of these products?

I am here today to urge FDA to finalize the guidance in a form that mandates that biologically-derived products intended for active wound healing are adequately regulated.

I would like to stress three points. First, it is appropriate for FDA to apply the terms “minimal manipulation” and “homologous use” narrowly. If not narrowly applied, Section 361 will continue to be a loophole through which unproven products will continue to be inappropriately marketed to clinicians.

The guidelines state that an HCT/P that is intended for use as an unproven treatment for “a myriad of diseases or conditions,” is “likely not intended for homologous use only,” I would urge FDA to delete the word “likely” and the reference to “myriad conditions.” To avoid any doubt, it is important that the final guidance states clearly that if an HCT/P is intended for use as an unproven therapeutic treatment for *any* disease or condition, it is not intended for homologous use. Therefore, it is not supposed to be regulated under Part 1271.

The types of data that are needed to support wound healing claims have been set forth in clear terms in FDA’s “Guidance for Industry on Chronic Cutaneous Ulcer and Burn Wounds — Developing Products for Treatment.” When FDA finalizes the homologous use guidance, the Agency should make clear that claims of therapeutic treatment require clinical trials according to established FDA regulations and guidance. Such products will generally not be considered to be homologous use products under Section 361.

Second, I urge FDA to clarify that homologous use does not include any function the tissue could *conceivably* perform in the donor. It is appropriate (and consistent with precedent) for FDA to limit the definition of “homologous use” of a tissue to the same basic functions that it serves in the donor. In particular, it is appropriate to hold, generally, that a homologous use of a tissue that is structural in the donor is limited to the same basic structural functions in the recipient.

Lastly, I would submit that initial assertions of homologous use and minimal manipulation in products should be defined by clear basic science. The current regulatory scheme allows incentives for immediate product availability without FDA premarket review, but not proof that the products continue to serve as anticipated in a homologous use role. The lack of meaningful oversight and clarity with respect to the scope of homologous use claims – combined with validation of such claims by payors – encourages marketing beyond the original scope of homologous use.

Thank you for the opportunity to comment on these guidance documents.

Sincerely,


John Samies, M.D.